

AHN West Penn Hospital

4800 Friendship Ave, Pittsburgh, PA 15224

DISCHARGE SUMMARY Document #: DIS-006

Patient & Admission

Name	Natasha Wood
Date of Birth	1946-05-06
Phone	878-555-5313
Address	Butler, PA, 16001
MRN	MRN-980841
Admit Date	2026-04-14
Discharge Date	2026-04-19
Attending	Nathan Phelps, MD
Attending NPI	8184514627

Insurance

Primary Payer	Medicare
Primary Member #	9HUQ-09N-1699

Primary Discharge Diagnosis

NSTEMI, mid-LAD distribution

ICD-10: I21.09, I25.10

Hospital Course

The patient presented to the emergency department with acute onset substernal chest pressure and dyspnea. Initial electrocardiogram demonstrated ST-segment depression in the anterior leads with reciprocal changes. Troponin I was elevated at 2.8 ng/mL, consistent with acute myocardial infarction. Coronary angiography revealed a critical stenosis in the mid-left anterior descending artery with TIMI II flow. The patient underwent successful percutaneous coronary intervention with placement of a drug-eluting stent to the LAD with restoration of TIMI III flow and excellent angiographic result. No complications were noted during the procedure.

Post-intervention, the patient was admitted to the coronary care unit for continuous monitoring and medical optimization. Transthoracic echocardiography on hospital day two revealed a left ventricular ejection fraction of 38 percent with anterior wall hypokinesis consistent with the acute infarction territory. B-type natriuretic peptide was mildly elevated at 285 pg/mL. The patient was initiated on guideline-directed medical therapy including aspirin, ticagrelor, high-intensity atorvastatin, metoprolol, and lisinopril. She remained hemodynamically stable throughout the admission with no recurrent angina, arrhythmias, or signs of acute decompensation. Troponin levels peaked at 4.1 ng/mL on hospital day one and trended appropriately downward thereafter.

The patient was discharged on hospital day five in stable condition with appropriate follow-up arranged. She will continue dual antiplatelet therapy for twelve months and has been counseled on medication adherence, dietary modifications, and cardiac rehabilitation participation. Outpatient cardiology follow-up is scheduled in two weeks to reassess left ventricular function and optimize heart failure medications as needed. The patient demonstrated good understanding of discharge instructions and expressed commitment to lifestyle modifications.

Procedures Performed

Coronary angiography	CPT 93458
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Medication Changes

Clopidogrel 75 mg daily	STARTED
Metoprolol succinate 50 mg daily	STARTED
Atorvastatin 80 mg daily	STARTED
Aspirin 81 mg daily	STARTED

Follow-up Instructions

Recommended Specialist	Cardiology
Follow-up Window	8 days
Urgency Tier	CRITICAL
Clinical Flags	EF<40%, recent-revascularization, post-PCI dual antiplatelet therapy

Dictated and electronically signed by:
Nathan Phelps, MD
NPI: 8184514627
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